

Veterans Health Administration

COVID-19 Operational Plan – Version 1.8

IMPORTANT NOTE:

VERSION 1.8 OF THE COVID-19 OPERATIONAL PLAN PROVIDES INTERIM FIELD GUIDANCE WHILE GENERIC RECOMMENDATIONS FOR VIRAL RESPIRATORY INFECTIONS ARE DEVELOPED. AN UPDATED RESPIRATORY ILLNESS SAFETY PLAN IS ANTICIPATED IN THE COMING MONTHS AND WILL REPLACE THE COVID-19 OPERATIONAL PLAN.



VA

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Document History Log

The VHA COVID-19 Operational Plan is a living document that will be updated to reflect new guidance and information. Any revisions to the Operational Plan will overwrite the baseline version. The full list of previous updates can be found in the [History Log](#).

Document Type (Baseline/ Revision)	Document Revision	Effective Date	Description
Revision	v1.8	03/11/2025	1. Removed the Communications section from the Executive Summary as these materials were outdated. 2. Updated the COVID-19 Operating Framework to offer Recommendations for Management of COVID Infection Rates. 3. Removed the section on Contingency Staffing Guidelines and will defer to CDC guidelines for healthcare settings. 4. Merged the Standard & Enhanced Safety Practices into one table for side-by-side reference. 5. Updated the Standard & Enhanced Safety Practices table: added the “back to work protocol” section with links to established VHA and CDC guidance. 6. Removed ED & Urgent Care, Open Bay Medical ICUs, and Long-Term Care SCI/CLCs from the areas where masking is required under standard safety practices 7. Eliminated Appendix A and B and incorporated guidance for Community Living Centers and Mental Health Acute Inpatient and Residential Treatment Units within the Internal and External Alignment section on page 4.

Executive Summary

The Veterans Health Administration (VHA) COVID-19 Operational Plan is iterative guidance providing the response framework and practices for all VHA facilities to proactively navigate the changing COVID-19 landscape.

Guiding Principles

Empowering Local Leadership

Consistent with the High Reliability Organization principle of Sensitivity to Operations, Veterans Integrated Service Network (VISN) and facility leadership discretion should guide individualized application of this guidance to meet unique local circumstances and needs (e.g., clinic-level differences, facility constraints, staffing shortages, or community outbreaks). Local leaders are best informed of and able to proactively manage their capacity and demand for in-house and community care. VHA Central Office clinical and operational leadership supports and empowers local leadership, informed by this plan, to make the right decisions to ensure Veterans receive safe, timely and clinically appropriate care.

Maximizing Veteran Access and Minimizing Staff Burden

The Operational Plan establishes a framework for facilities to continue providing Veterans with optimal access to safe, timely, high-quality care despite increases or decreases in COVID-19, influenza, and other viral respiratory infections. Facilities should use this guidance to prioritize Veteran and employee safety and offer a full spectrum of flexible care options to meet Veterans' needs. Facilities should promote and sustain employee resiliency and minimize changes in working conditions as much as practical to meet their needs in the face of varying COVID-19 conditions.

Internal and External Alignment

The Operational Plan provides general guidance for VHA facilities where healthcare is delivered, including Community Living Centers (CLC) and Mental Health Inpatient and Residential (Domiciliary) Treatment Units. The Operational Plan is in alignment with COVID-19 infection control guidelines for healthcare settings from Federal partners, including the Centers for Disease Control and Prevention (CDC) and the Occupational Safety and Health Administration (OSHA). Additional considerations for the Community Living Centers (CLC) are outlined within [CDC's Infection Control guidance](#) under "Setting-Specific Considerations". VHA's [Risk Assessment Tool for CLC/SCI-D Admissions during COVID-19 Outbreak Investigation](#) provides additional population specific framework for decision making. Specific guidance for Mental Health Acute Inpatient and Residential (Domiciliary) Treatment Units can be found at [Mental Health Acute Inpatient and Residential COVID-19 Resources \(sharepoint.com\)](#).

[Guidance for healthcare settings](#) may differ from more general respiratory illness [guidance for non-healthcare settings](#). VHA facilities may have areas of a campus that fall under different recommendations due to their unique configuration.

Recommendations for Management of COVID-19/Viral Respiratory Illness Based on Changing Local Epidemiology

This Operational Plan consists of a set of Standard Safety Practices that should be implemented at VHA healthcare facilities at all times. In addition, this plan provides VHA healthcare facilities with a menu of Enhanced Safety Practices in order to respond, at a unit or facility-wide level, to changing epidemiological conditions. Some triggers may include rising community levels of respiratory viral activity (“waves” or seasonal variations), outbreaks within well-defined work areas, higher than usual bed occupancy due to viral respiratory illness, or infection related staffing challenges.

The local hospital epidemiology and infection prevention staff are subject matter experts (SMEs) to advise the facility’s executive leadership on additional actions to take during a COVID-19/viral respiratory illness outbreak. When there are changes to a facility or unit’s safety practices, these may be memorialized in committee minutes or memos sent to hospital staff.

Special consideration should be given to high-risk units (chemotherapy units, dialysis units, transplant units), with a low threshold to implement broad masking requirements and other Enhanced Safety Practices in response to local outbreaks, increasing local community respiratory viral activity as determined by local hospital epidemiology or infection control staff, or at the direction of public health authorities or VHA national program offices.

VHA's COVID-19 Operational Guidance on Standard and Enhanced Safety Practices

	Standard Safety Practices	Enhanced Safety Practices
Masking & PPE	• In most instances, masking is optional. • Masks must be readily available to all patients, staff, and visitors. • Masking is required for staff, patients, and visitors while in the high-risk areas listed below, including waiting areas: ○ Transplant units ○ Dialysis ○ Chemotherapy units Individuals may remove masks in these high-risk areas under the following circumstances: ○ They are not in a patient room ○ They are not within six feet of a Veteran in a congregate area ○ They are not within six feet of a patient in an open treatment area/room • Masking is also required: ○ For staff, patients and visitors who have suspected or confirmed SARS-CoV-2 infection or other respiratory infection (e.g., those with runny nose, cough, sneeze); or had close contact with (patients and visitors) or a higher risk exposure (e.g, health care personnel, (HCP)) to someone with a viral respiratory infection, for at least 10 days post-exposure.	• Re-institute broader masking protocols in consultation with local SMEs (facility-wide or confined to specific unit/work areas). A graduated approach to masking may also be considered such as masking only during clinical encounters¹ or in clinical areas. • Provide N95 respirators and/or eye protection for direct care of patients without known or suspected COVID-19, in all or some areas

	○ Where local leadership determines masking is required. • During a clinical encounter¹ VHA HCP should wear a mask upon request of a Veteran patient, caregiver or family member. Any inability or concern to comply with such masking requirements should be reported to the supervisor. • Follow current OSHA standards for employee PPE requirements. • N95 respirators must be provided to any staff member who prefers to wear one while on duty (even if not required) on a voluntary basis • Care for COVID-19 patients requires use of N95 or above^{2 3} along with other PPE (e.g., gown, gloves, eye protection).	
Visitors	• Facilities should allow visitors including family members, caregivers, volunteers, vendors, surveyors, and others, subject to their normal visitor protocols. • There may be visitation restrictions for persons with known or suspected COVID-19 or other respiratory infection. • Certain high-risk areas might institute visitation policies that restrict the hours, volume, or conditions of visitation to consider therapeutic benefit but also ensure the safety of Veterans and other patients such as roommates. Some of these clinical areas may include the emergency department (ED), acute inpatient	Consider implementing the following: • Limit number of essential companions • Encourage use of alternative mechanisms for patient and visitor interactions such as outdoor visitation, video call applications, designated visitation rooms • Limit time of visitation • Pre-schedule visits • Limit movement of visitors among other parts of the medical center Special consideration should be made for patients with children, end of life care/compassionate care, and individual patient therapeutic needs.

¹ Clinical encounter refers to an individual in-person encounter, between a patient and health care personnel, during which a clinical assessment or clinical activity is performed.

² See OSHA regulation, [29 C.F.R. § 1910.134, regarding respiratory protection requirements in workplaces.](#)

³ [CDC PPE Guidance](#)

	units, inpatient mental health and residential units, CLC, and other congregate settings.	
Testing	COVID-19 Testing Guidebook	
Care Balancing	All care is provided using modalities (virtual or in person) most appropriate to the needs of the Veteran	Consider implementing the following: • Shift clinical encounters to virtual modalities when clinically appropriate. • Referring care to the community should only be considered as a last resort when staffing and bed capacity are unable to sustain demand, and with strong consideration to the community facility's safety practices. • Provide virtual alternatives to in-person group activities considering Veteran preferences, clinical benefit, and infection risk. • Implement surge capacity staffing plans. • Reference VHA's Risk Assessment Tool for CLC/SCI-D Admissions during COVID-19 Outbreak Investigation
Physical Distancing	No physical barriers or distancing requirements	Consider implementing the following: • <i>Lower occupancy of waiting areas to maintain ≥6 feet physical distancing where practicable.</i> • Install physical barriers in lobbies and check-in areas
Workplace	No restrictions on meetings or travel.	Consider implementing the following: • Discourage indoor in person meetings unless mission critical. • Move meetings to non-healthcare buildings. • Utilize virtual meetings where feasible.

Ventilation	- New construction or renovation should incorporate any enhanced ventilation requirements and follow the VHA design guide.⁴ - CDC Infection Control Guidance: SARS-CoV-2	Consider implementing the following: - Increase ventilation to COVID surge areas. - Use portable HEPA-filter air scrubbers
Cleaning & Disinfection	HEFP COVID Cleaning Matrix CDC Guidance: When and How to Clean & Disinfect a Facility CDC Environmental Infection Control Guidelines	Consider increasing frequency of cleaning in common areas/high touch surfaces and high-risk areas.
Back to Work Protocol	Interim Guidance for Managing Healthcare Personnel with SARS-CoV-2 Infection or Exposure to SARS-CoV-2 COVID-19 CDC Strategies to Mitigate Healthcare Personnel Staffing Shortages COVID-19 CDC OPM Memorandum on Leave Policies Related to COVID-19 Vaccinations and Boosters for Federal Employees	

⁴ Office of Construction & Facilities Management Design Manual: <https://www.cfm.va.gov/til/dManual.asp>